

Basic communication skill



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Outline



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Lecture learning outcomes



- Outline the importance and characteristics of communication in healthcare & barriers of communication
- Describe the key skills for communicating about health behavior change

Introduction



- Individuals often feel more reluctant to think about change if they perceive their freedom is being compromised. So perhaps if the clinician is able to create an environment of understanding and respect for the patient's autonomy, a patient may feel more comfortable in discussing health behavior change.
- Good, skillful communication is the key to creating this kind of environment.





- Communication is a process by which we allocate and convey meanings in an attempt to create shared understanding with others.
- The choice of language, tone of voice, non-verbal cues, and gestures we use are important in ensuring the right message is conveyed.



Types of Communication

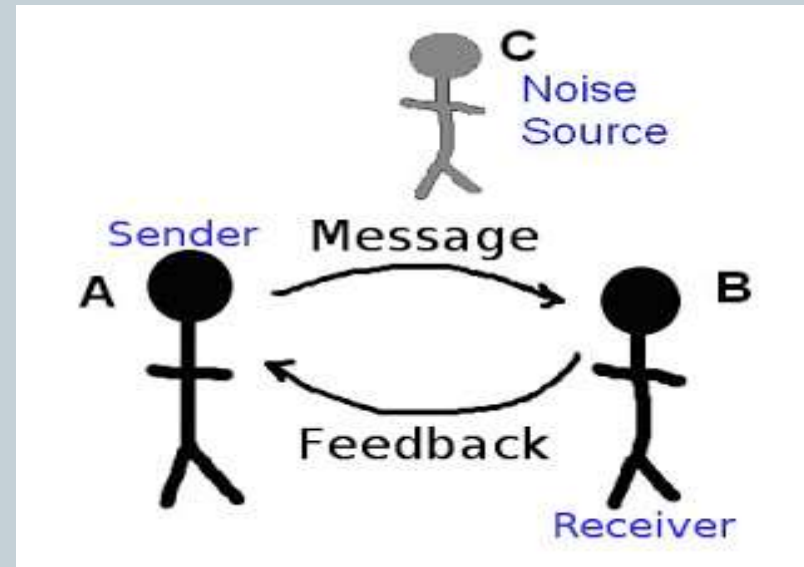


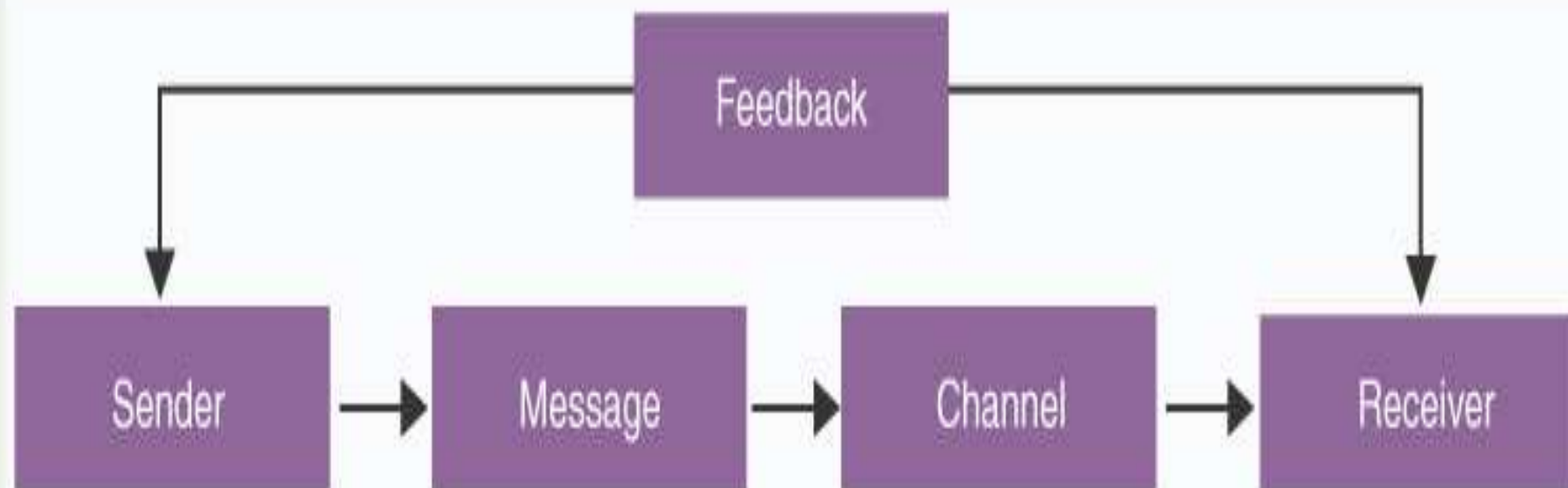
1. Verbal: Style of conversation, Matter of conversation, Choice of words, Voice control.
2. Non-verbal: Involves a whole range of bodily movements and facial expressions like smile, raised eyebrows, frowning.



Components of Communication

1. Sender
2. Receiver
3. Message
4. Channels of communication
5. Feedback





- Communication skills
- Attitudes
- Experience
- Culture
- Self-concept

- Elements
- Structure
- Content
- Treatment
- Coding

- Seeing
- Hearing
- Touching
- Smelling
- Tasting

- Communication skills
- Attitudes
- Experience
- Culture
- Self-concept



- If the dental clinician can accurately select the most appropriate **style of communication** to employ in a given consultation, and utilize communication skills that make the **patient feel respected** and in control of any changes to be made, this will create a setting where a **productive discussion** about change can take place.



THE RELATIONSHIP BETWEEN THE CLINICIAN AND THE PATIENT

- The first initial step in any consultation about health behavior change should be to create a good “ rapport ” or “ constructive relationship ” with the patient.
- Rapport is important to consider, as the quality of the relationship between the clinician and the patient correlates with patient behavior change outcomes.



Ideas on how to create a good rapport with your own patients:



- Smile and welcome patients into appointments
- Think about the non - verbals
- Think about when you ask questions
- Take some time to listen to patients
- Make them feel comfortable enough to come back in the future





- **Selecting the right style of communication** for a given consultation is often the key to creating and maintaining a good rapport.

STYLES OF COMMUNICATION

- Clinicians might communicate with their patients in daily practice, using either 1) directing, 2) guiding, or 3) following style.

Three styles of communication

Directing

Teaching
Instructing
Leading

Guiding

Drawing out
Encourage
Motivation

Following

Listening
Understanding
Going along

3 Conversational Styles



Directing

Following



Guiding

Directing

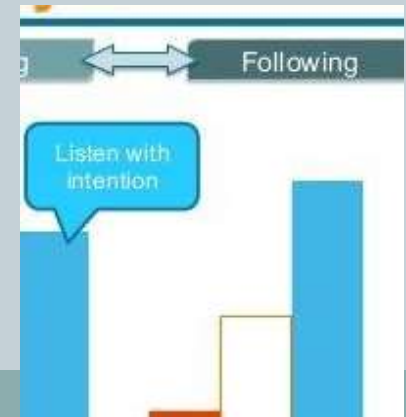


- *A directing style involves the delivery of expert advice and help.*
- This has traditionally been the dominant approach within health care settings.
- Directing is best employed where there is a good rapport between the clinician and the patient.

Following



- A *following style* utilizes **listening** skills and occurs in situations where sensitivity is required (such as when a patient is upset).
- The goal of a clinician using a following style is not to solve the patient's problem immediately but to provide **support** and encouragement (for example, when breaking bad news to a patient).



Guiding



- In guiding, the clinician and patient work together to help the patient identify his or her own goals and how he or she might best achieve them.
- This style is most appropriate when talking to patients about making health behavior changes — especially with those who may be ambivalent about changing.





- Skilled clinical communication is about being able to effectively identify which styles of communication are best suited to a given point during the dental visit and being able to switch flexibly between those styles during the interaction.



A helpful exercise to consider which style might be more appropriate (directing, guiding, or following) is provided via the examples below:

“What can I do to stop needing a filling every time I come back here?”
(Directing)

“I’m really afraid that I’m going to lose my teeth.” (Following)

“I can see how flossing might help my gums, but I just find it so difficult to fit in.” (Guiding)

“It would be really helpful to know what kind of foods I should avoid.”
(Directing)

CASE



- A 65 - year - old man receiving treatment for periodontal disease. This man is mostly compliant with his oral hygiene routine and attends his appointments regularly. However, despite repeated information and advice given to him by his dental clinician and others, he continues to **smoke** forty cigarettes a day. He is also slightly overweight and, on assessment, admitted he had a **high sugar diet** with little intention of changing this.
- Consider the six questions when you are communicating with the him about his health behaviour change.... How?

Six broad questions in relation to communicating about health behavior change



- **Q1: Is the patient happy to talk with you about behavior change?**
- **Q2: How are you asking questions?**
- **Q 3: How do you provide support?**
- **Q4: How do you convey understanding?**
- **Q 5: How do you get information across?**
- **Q 6: How do you bring it all together?**

1. Is the patient happy to talk with you about behavior change?



- Deciding to talk about behavior change should ultimately be the patient's choice, to increase his or her feeling of personal freedom to make decisions, and in turn lower resistance.

2. How are you asking questions?



- Open questions encourage the patient to explore how he or she feels about a particular behavior. They help the clinician to gain a deeper understanding of the patient ' s views on change.
- Closed questions only give limited information, which makes it harder for the clinician to deduce information that may be clinically relevant.

3.How do you provide support?



- Affirmations that demonstrate appreciation for the patient ' s efforts, can be useful in helping the patient believe that he or she has the ability to change/ reinforce positive acts.

4. How do you convey understanding?

Reflective Listening



- Listening is generally a passive process — the listener remains quiet and hears what the other person is saying.
- However, unless the listener checks what he or she has understood, it cannot be assumed that the listener accurately understands the other person 's meaning.
- Listening needs to be an active process.
- Reflective listening involves making a statement in reply to what the patient has just said in order to demonstrate understanding.



TYPES OF LISTENING

2. Reflective listening

To clarify what you've heard

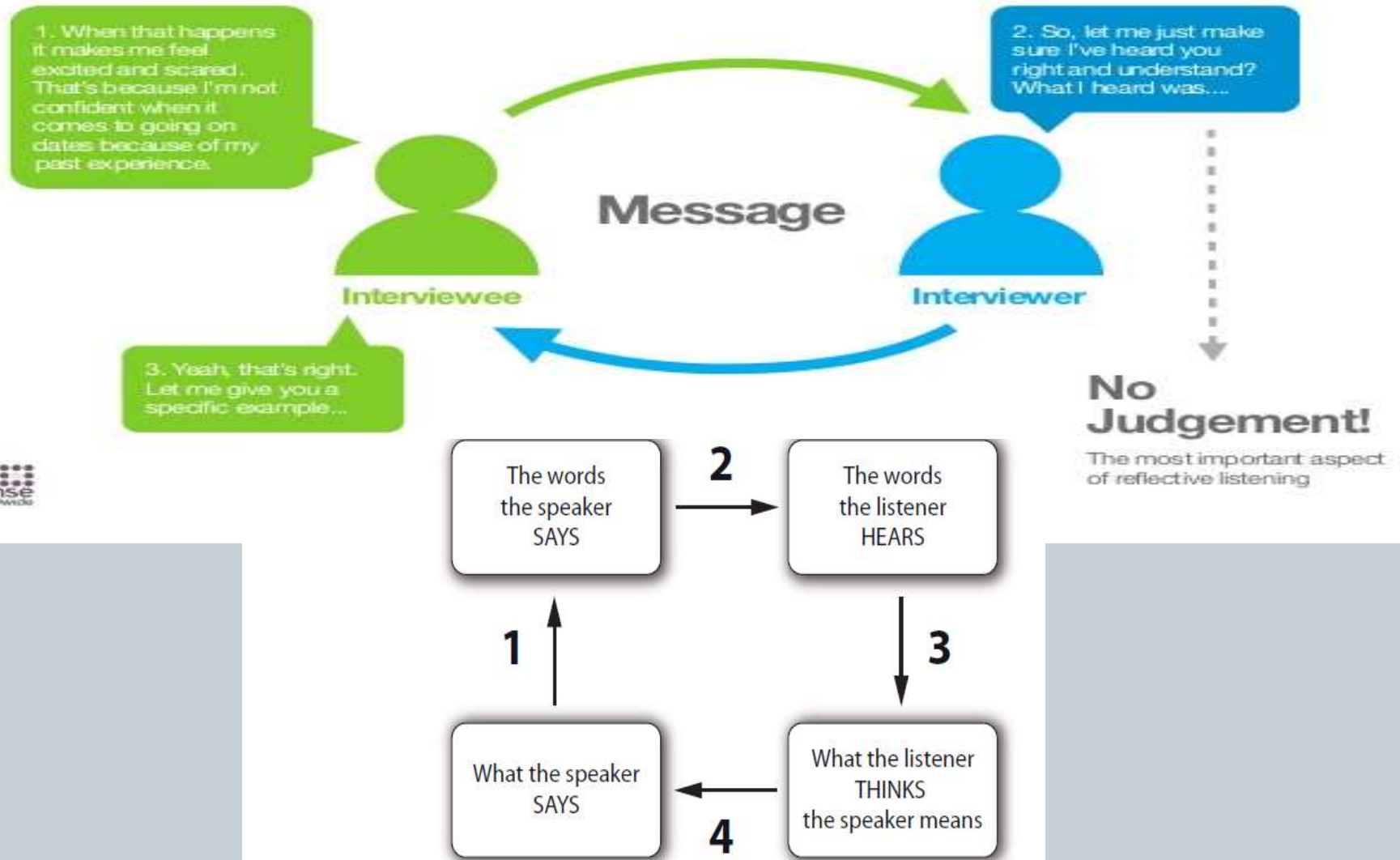


Figure 3.2. Reflective listening.

Process of reflective listening works..



- Reflective listening serves two functions:

Firstly, it enables the listener to check what he or she has understood from what the speaker has said.



Secondly, it enables the speaker to feel understood (in turn enhancing the rapport between the two parties), and it also encourages the speaker to elaborate and give more detail.

Q:5. How do you get information across?



- Redundant information, time is precious; thus, telling the patient things that they already know is a waste of that time.





When information needs to be conveyed to the patient, in order to be effective:

- The information needs to add to the patient's existing knowledge and it needs to be seen as personally relevant by the patient.
- One way in which the clinician can seek to achieve this is through the elicit - provide - elicit method.



Elicit - Provide - Elicit Method



Assess what the
patient already
knows



Provide further
information



Elicit what the
information
means for the
patient

Q 6.How do you bring it all together?



- Summaries are useful tools for the clinician to consider what the patient has said and to consider how the patient feels about making particular changes.

Summaries



1. A collecting summary (*brings together key issues raised by the patient*)
2. A linking summary (*links together what the patient has said about the topic*)
3. A wrapping - up summary (*to close a session, or move the discussion on*)



Table 3.1. Summary of OARS.

Skill	Description	Example
Open question	A way of eliciting greater patient response to a request for information	“How do you feel about ...”
Affirmation	A way to build patient confidence and enhance rapport by recognizing the positives	“You can see you did really well ...”
Reflective listening	A way to express understanding and validate the patient while facilitating increased discussion and patient engagement	“It sounds like you ...”
Summarizing	An empathic way to sum up, link different parts of the conversation, or wrap up	“So let me see if I have this right, so far you have said that ...”
Exchanging information	A way to find out what the patient already knows and to increase their knowledge in a way that actively engages them in the process	“What do you know about ...”

Barriers in Communication

Psychological
barriers

Physiological
barriers

Environmental
barriers

Cultural
barriers

References



- Chapters 3: Ramseier C, Suvan JE, editors. Health behavior change in the dental practice. John Wiley & Sons; 2011 Jun 9.
- Chapters 18: Mostofsky DI, Fortune F. Behavioral dentistry. John Wiley & Sons; 2013 Dec 31.



Thank You